

Code Yellow

How rapid experimentation outperformed Epic and saved a \$25M client

Fact block

Company	Cedar
Role	Lead designer
Timeline	5 months
Team	Product, Engineering, Data Science, Client Management, Legal
Stakes	\$25M in revenue
Key metric	Collections rate

Headline metrics

+24% collections rate, outperforming Epic **25+** experiments shipped in 5 months **\$75M** in revenue secured — a 3-year, \$25M/year renewal

The situation

Cedar's biggest client threatened to churn, triggering a performance bakeoff against Epic, our biggest competitor. I led the design response.

The bakeoff was measured two ways. First, the client paused Cedar at one location and ran a before/after comparison there. Before they paused us, we had a month to rapidly launch experiments and boost our performance baseline going in. Second, they compared that paused location against a sister location that kept Cedar on. That second comparison is where I focused most of the experimentation, since it was the more direct head-to-head against Epic.

Identifying the opportunity

A funnel analysis from Data Science showed where patients were dropping off: roughly 93% of all drop-off happened before checkout, not at it. This pointed to an opportunity to improve top-of-funnel comms in order to get more patients to engage with their bills and pay.

Within that group, insured patients with large bills stood out. They made up just 16% of the patient population, but 46% of annual revenue. That split made them the highest-leverage place to focus.

Design approach

We had a goal of impacting/learning from patients 1x a week. This speed was made possible by an internal tool I previously designed that enabled us to send one-off email to targeted patient lists. Each email took a few days to launch vs. the typical 2-sprint eng timeline for emails. These emails were much more simplistic than our typical emails (ex: most text-based, limited patient data we could pull in), but they allowed us to quickly test new ideas before investing in eng build.

We launched 25+ experiments shipped in 5 months. Two are worth walking through in depth.

Experiment #1: Partial pay → payment plan nudge

Hypothesis: Patients pay part of their bills and then drop off. If we can get them to sign up for a payment plan, we can expect to see more balances fully paid off.

Started with a minimal viable test

This idea actually came from the client. To establish credibility fast, we launched a simple one-off email to patients with outstanding balances who'd made a partial payment in the last month educating them on payment plans.

Analyzing results

After 30 days: 5.28% payment plan creation in the test group vs. 3.27% in control. Not stat sig, but a positive signal across both email engagement and payment plan creation gave us the greenlight to keep going.

Two things stood out in the data: Partial payment amounts tended to land within the monthly installment range we were already offering, and 65% of partial payers clicked into the payment plan option sometime between their latest invoice and their latest payment. This told us these

patients were interested in payment plans and likely could afford the monthly installment amount.

Scaling with client buy-in

I got buy-in to nudge partial payers at more points across their journey: In addition to nudging patients towards payment plans after making a payment, I saw an opportunity to educate them on payment plans earlier when initially making the payment. From there, I worked with engineering to make the payment plan recommendations dynamic based on the amount a patient entered, instead of one static suggestion. Most patients' partial payments fell within a normal monthly installment range, so we could suggest a plan that matched.

For the emails post-payment, I also worked with engineering to make these emails dynamic. When the partial payment unlocks lower payment plans, we wanted to highlight the affordability of the new plans.

Results

The final version of the nudge held up: +18.8% payment plan conversion and +4.2% overall payment conversion, both stat sig. We rolled it out as a general enhancement for all clients.

Experiment #2: Flexible payment options

Hypothesis: Patients put off paying because they aren't sure which option they can afford. If we recommend specific payment options we think will work for them early on, we can expect more patients to pay off their balance.

Understanding insured patients with large bills

Data Science had already flagged this segment as the opportunity. I connected it to research our UXR team had done on ALICE patients — Asset-Limited, Income-Constrained, Employed. This growing population of patients in the US make above the poverty line, so they don't qualify for financial assistance. However, they still can't pay off their medical bills. One patient in the research described maxing out credit cards and refinancing a house just to keep up with medical costs.

This reframing of these patients inspired me to design our comms to be being less transactional (“you owe this”) and more supportive (“here are options that may work for you”).

From brainstorm to concepts

I ran ALICE x Comms brainstorm sessions with the cross-functional project team and the Design team, working from the question: how might we personalize these communications so patients feel more supported in managing and paying their bills? That produced a handful of directions—emphasizing payment plans, skip/pause options, bill prioritization, empathetic messaging—that we narrowed down and partnered with UXR to test with ALICE lookalike users across five early concepts.

Turning a research insight into an experiment

One insight from that research stuck with me: patients wanted to know their options as early as possible, even if they didn't need them yet. One patient put it simply — she liked having options, even if she had the money upfront, because she wanted to know what was available to her in the future.

That became the experiment: a "here are a few ways to pay your balance" email, laying out flexible options before a patient ever had to ask.

Refining and scaling based on the MVT

The first version went to patients with balances over \$500 and multiple bills, sent as a one-off email before any bills were past due. Once we saw a signal, I refined the targeting to balances between \$500–\$2,500 with either multiple or single bills, and moved the timing to 7 days after the first bill reminder. Additionally, I made the recommended payment options dynamic based on balance size and number of bills.

The result: a 15.7% increase in payment rate across all balances, concentrated most in the \$500–\$2,500 range — a segment that's historically hard to collect on. The most common options patients chose were payment plans (+6.5%) and partial payment (+8.6%).

We shipped four final versions of the email, each tuned to a different patient situation: large balance/single bill, large balance/multiple bills, small balance, and medium balance.

Results

The final version drove an 8.24% relative increase in collection rate. We rolled it out more broadly to all providers, and added an SMS version alongside the email.

Enabling the broader team

Beyond my own experiments, I built infrastructure so other designers could run experiments with fewer blockers:

- A designer's manual for comms personalization

- A Claude skill for email copywriting, encoding the voice and structural rules other designers needed to write on-brand patient comms fast
- A legal x data guidance doc, clarifying what patient data was safe to reference in messaging and where

The outcome

Cedar outperformed Epic by 24% in collections rate. The client renewed for 3 years at \$25M a year — \$75M in total revenue secured. Across 5 months, 25+ experiments shipped, proving that thoughtful, low-code design interventions can move a business outcome this size.
